FOR THE HAVEN PAEDIATRICS BOARD

# What to expect over the next six months

## The mandate, the milestones, who does what, and what we need from you

**Date** 18 September 2026
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**Covers** October 2026 to March 2027

The audit is finished and the strategy is agreed, so this note sets out what happens now, what you should expect each month, and what the programme needs from the board in order to land.

## The mandate

Haven turns over between fifteen and twenty million naira a month today. The target the board set is forty-five to fifty million a month at an earnings margin of eighteen to twenty per cent within six months, led by the neonatal unit and timed into the paediatric high season which runs from November to February.

I want to be honest about what that number means, because six months reaches it at the February peak rather than holding it every month, and holding it year round is a longer build that runs past this programme. If neonatal referrals come through as planned, February lands in the target range. If referrals lag, February lands closer to thirty-eight to forty-two million, which would still be roughly double where Haven is today.

## The five jobs

| Job | What it means | The levers |
|---|---|---|
| **Fill the beds** | Occupancy is the fastest money, and the neonatal cot is the highest-yield bed | Referral portal and a dedicated analyst calling hospitals, a hyperlocal neonatal growth plan, more insurer and corporate clients |
| **Lift value per patient** | The priced lines in your tariff average thirty-four per cent below competitors | Reprice the tariff in two tranches, expand services including surgery, the Haven Club, and add-on services |
| **Free cash and margin** | Cash is tied up in stock and in amounts owed by payers rather than missing | Vendor-managed inventory, and a decision on laboratory outsourcing |
| **Build the engine** | Nothing above holds if the workforce keeps turning over | Cultural transformation, the people systems the audit found missing, and a staff application to run them |
| **Make Haven known** | Reputation is already strong with families and weak with referrers | Events, patient-centred design, podcasts, and the existing digital agency held to leads rather than impressions |

## What you should expect to see, month by month

| Month | What lands | How you will know |
|---|---|---|
| **October** | Governance instruments live. Incident register open. Departmental objectives published. Tariff tranche one. Referral portal and analyst in post. Operations recruitment under way | First monthly pack, new format |

| Month | What lands | How you will know |
|---|---|---|
| **November** | Referral programme running into the high season. Haven Club launched. Insurer and corporate pipeline building. Staff application in use. First clinical audit | Referral volumes by source, weekly |
| **December** | Peak begins. Working capital released through vendor-managed inventory. Laboratory outsourcing decision taken | Cash conversion in the monthly pack |
| **January** | Tariff tranche two. Service expansion including surgery. Add-ons live | Revenue per encounter against baseline |
| **February** | Peak month. The target is measured here | The number, against forty-five to fifty million |
| **March** | Hold the gain. Systems handed to the senior operations leader | Second maturity assessment, against 2.5 today |

## Who does what

We build the instruments, the registers, the pathways, the reporting and the referral programme, and we draft every document that needs approving, so that the work arrives at your door already done. You approve it, sign it, and use it, because a document we build that nobody adopts is worth nothing. Two things sit only with you: the Medical Director attends the referral meetings that matter and hosts the unit tours, which cannot be delegated to anyone without a clinical licence, and the board makes the senior operations appointment and gives it a real mandate.

## What we need from the board

Four things, and three of them cost nothing. The first is the operating principles that settle how far Haven invests in its workforce before it presses for margin, because today the answer depends on which owner is asked. The second is the senior operations appointment, which the whole board already supports and which most of this programme depends on. The third is the pricing decision, taken once and held, because a tariff rise reversed under pressure is worse than one never attempted. The fourth is your time at the strategy session and the fortnightly review, which is the only way this stays yours rather than becoming ours.

> **One thing worth saying plainly. Your families rate you 4.60 out of 5 and your staff rate the safety of the care they give as very good or excellent in twenty-two cases out of twenty-three, so the hard part is done and none of this is about fixing your clinical care. It is about building the machinery underneath it so the standard survives growth.**

The natural first step is the strategy session, which should hold before the November season opens, and I would like all five founders in the room for it.

Warm regards,

**Dr Debo Odulana**

Founding Partner, Consult for Africa